

One platform, three domains, three editions

The clinical record, operational flow and board assurance on one open core — so an incident recorded on a ward becomes a risk on the board’s register without anyone re-keying it.

Capability map · counts generated from the platform’s own edition definition · commvita.com

● Live — API-backed, wired end to end

● Demonstrated — seeded data

Status is stated per module, never per platform

Why this matters

Primary care, community, mental health, social care and the acute trust each run separate systems. The person is scattered across all of them and whole in none.

When systems don’t connect, people carry the data between them — the most costly and error-prone integration layer there is.

Three domains, one record

- **Clinical** — the record and the care, across every setting a person passes through
- **Operational** — flow, capacity, discharge and the workforce that runs them
- **Governance** — incident to board, risk, information governance, AI, audit

Same person, same data, three lenses — so a decision in one is visible in the others.

Key message

Open standards underneath, so the data stays yours. Self-hosted or hosted, configured per jurisdiction, and an export you can check instead of trust.

Editions are additive: start with the part of the problem that hurts, add the rest without re-implementing anything.

What isn’t there

A page of badges would be easier to print than this list, and worth less.

- No ISO 27001, no Cyber Essentials
- No Data Security and Protection Toolkit return submitted
- No certification for the record-sharing standards
- No independent penetration test or disaster-recovery evidence published
- openEHR-*aligned*, not native: it writes to an openEHR repository, ships **0 archetype definitions** and has no AQL

End to end, in six moves — data arrives, becomes a record, runs the operation, proves itself, and teaches the system

1

Source systems

Hospital and GP systems, community, mental health, social care, pharmacy, ambulance, registries and devices — each staying where it is.

EPR · GP · LIMS · registries

2

Exchange & assurance

Monitored connectors translate feeds to one canonical shape, and report what is delivering, how fresh it is and what was rejected.

/connector-dashboard

3

Meaning & identity

Coded to published vocabularies, resolved to a person with a score on the match, and written to an openEHR repository.

/patients · /empi

4

Run the operation

The health economy as one production line: the binding constraint found from the model, then acted on across acute, community and social care.

/flow · /discharge-hub

5

Govern & evidence

Event to theme to systemic issue to risk to board objective, each promotion carrying its evidence, on a hash-chained audit.

/golden-thread · /audit

6

Learn & improve

Population segmentation, inequality gaps, and federated cohort discovery that returns counts while the data stays at source.

/population-health · /omop-cdm

Three editions, additive — every capability group, with the number of capabilities in it

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CAPABILITIES · 21 GROUPS

commvita Flow

The operational base. Data, integration and the operational hub, with parity against the national data platform. No clinical record.

- FDP-parity operational products 19 Interop 19 Platform 18
- US-ACO 13 Core utilities 10 Open core platform 9 Analytics 7
- Operations 7 Finance 6 Developer toolkit & ecosystem 4
- FDP transition & data model 4 Patient-facing 4 Shared care 4
- Integrations 3 Access 2 Identity 2 Operations & flow 2
- Planning 2 Referrals 2 Data 1 Workforce 1

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ADDED · 16 GROUPS

Governance & Assurance

The edition boards buy. The organisation’s governance system of record and its organisational memory. Carries no clinical workflow at all.

- Incident management 14 Information governance & privacy 12
- Training, CPD & workforce assurance 12
- Statutory readiness & operational command 11 Risk & compliance 10
- Audit & evidence 10 Board assurance 9 Workforce 9
- Data & AI governance 7 Governance intelligence 7
- Statutory reporting & annual returns 7 US-ACO 3 Analytics 2
- Ambulance 1 Clinical 1 Operations 1

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ADDED · 28 GROUPS

Population Platform

Where you end up. The clinical record across every setting, plus the population layer that reads it. Includes both editions above in full.

- Clinical 35 Mental health 14 Community & remote 12
- Social care 10 US-ACO 9 Ambulance 8 Community 7
- Finance 7 Care settings on the one record 6 Clinical record 5
- Population health 5 Patient-facing 5 Referrals 5
- Clinical operations 4 Clinical & national returns 4 AI / clinical 4
- Medicines 4 Analytics & AI 3 Analytics 3 Access 2 Platform 2
- Prescribing 2 Governance 1 Maternity 1 Operations 1
- Quality 1 Remote 1 Safety 1

The standards it is built to — named in text, because these are other bodies’ marks and we have no licence to print them

RECORD & TERMINOLOGY

- openEHR record shapes
- SNOMED CT dm+d
- PRSB Core Information Standard

EXCHANGE

- FHIR R4 HL7 v2 GP Connect
- NRL IPS
- EEHRxF — design alignment

SECONDARY USE

- OMOP CDM v5.4
- OHDSI network node
- FHIR Bulk Data Access
- k-anonymity
- ICO anonymisation guidance

SAFETY & CLINICAL RISK

- DCB0129 DCB0160
- Non-SaMD throughout
- NEWS2 (RCP 2017)

PRIVACY & ACCESS

- UK GDPR Art. 6, 9, 15, 20, 30
- National data opt-out
- FHIR Consent OIDC / SSO
- ODS organisation codes

INCLUSION

- Accessible Information Standard DCB1605
- NHS Reasonable Adjustment Flag
- Core20PLUS5 splits by default

The base underneath all three — you don’t buy it separately and you can’t end up without it

Identity & RBAC

Tamper-evident audit

Jurisdiction profile

Data fabric & open APIs

Person spine

Hosting

- None of it is hard to get. It just isn't done, and a first customer would be right to make it a condition of going live.
- A role decides what you can do; org scope decides whose records you see. Enforced on the server.
- Append-only, hash-chained, re-verified on read — so the log gets checked instead of trusted.
- Regulator, identifier scheme, professional register and legal deadlines, dated and source-cited.
- One canonical shape, a machine-readable contract, and bulk export that leaves in a published standard.
- Identity resolution that carries a score, so “probably the same person” stays visibly probable.
- commvita-hosted, self-hosted or sovereign. Flow Edition is £1 per instance, every environment included.